SYNTHETIC DOCUMENT. NOT A REAL PATIENT. FOR DEMO AND TESTING ONLY.

Northbrook General Hospital

Respiratory Medicine · Heron Ward (Ward 9)
1 Example Road, Northbrook NB1 0AA (fictional)
Switchboard: 0000 000 000 · Ward: 0000 000 009 (fictional numbers)

DISCHARGE SUMMARY

Our ref: RM/SYN-0003/22-10
Date: Thursday 22 October 2026

To: Dr B. Example, Riverside Health Centre, 8 River Walk, Northbrook NB4 0CD (fictional) **cc:** Patient

Patient:	Mr Kevin Reed	**Hospital no.:**	SYN-0003
Date of birth:	22 August 1996 (age 30)	**Address:**	Flat 3, 5 Test Lane, Northbrook NB4 0QQ (fictional)
Admitted:	Monday 19 October 2026 (emergency, via the Emergency Department)	**Discharged:**	Thursday 22 October 2026
Length of stay:	3 days	**Discharged to:**	Home, lives with his partner
Consultant:	Dr M. Sample, Consultant Respiratory Physician	**Allergies:**	**Ibuprofen and aspirin (cause wheeze)**

DIAGNOSIS

Asthma attack, likely set off by a cold.

TREATMENT IN HOSPITAL

Nebulisers, steroid tablets and oxygen for the first day. Kevin improved quickly. On the day of discharge his breathing was comfortable and his peak flow was 400 L/min. His best peak flow when well is 450 L/min.

MEDICINES TO TAKE AT HOME

Medicine	How to take it	Status
1. Prednisolone 5 mg tablets.	Take 8 tablets (40 mg) together once a day in the morning with breakfast for 5 more days. Last dose on Tuesday 27 October 2026.	New
2. Fostair 100/6 inhaler.	Take 2 puffs twice a day (morning and evening), every day, even when you feel well. Rinse your mouth with water after use.	Changed
3. Salbutamol 100 microgram inhaler (blue).	Take 2 to 4 puffs with your spacer when needed for wheeze or breathlessness, up to every 4 hours.	Continue
4. Omeprazole 20 mg capsules.	Take 1 capsule once a day in the morning while you are taking prednisolone.	New
5. Clenil Modulite 100 inhaler.	Stop. Fostair replaces this inhaler.	Stopped

Do not take ibuprofen, aspirin or other anti-inflammatory painkillers. Paracetamol is safe for you.

DAILY ACTIVITIES

- Peak flow: measure it twice a day, at 8:00 am and 8:00 pm, before your inhalers. Write the best of 3 blows in your asthma diary.
- Check your inhaler technique with the video link on your asthma leaflet.
- Gentle activity is fine. Build up slowly over the next week.
- Return to work: Monday 2 November 2026, if your peak flow is above 400 L/min.

DIET

- No special diet. Take your prednisolone with food.

THINGS TO AVOID

- Do not smoke, and avoid smoky places.
- Avoid strong perfumes and cleaning sprays while you recover.
- Do not run or do hard exercise until your GP asthma review.

FOLLOW-UP APPOINTMENTS

- GP asthma review within 2 working days. Please call Riverside Health Centre to book this.
- Asthma nurse clinic: Wednesday 18 November 2026, 11:00 am, Respiratory Outpatients, Northbrook General Hospital.
- Flu vaccine: at your GP surgery once you are well.

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WARNING SIGNS

Call your GP or Heron Ward on 0000 000 009 (fictional number) the same day if:

- You need your blue inhaler more than every 4 hours.
- Your peak flow is below 340 L/min.
- You wake at night with cough, wheeze or a tight chest.

Call emergency services straight away if your blue inhaler is not helping, you are too breathless to speak, your peak flow is below 225 L/min, or your lips are turning blue.
While you wait, sit up straight and take 1 puff of your blue inhaler every 30 to 60 seconds, up to 10 puffs.
Emergency services number: **555-555-0199** (fictional number)

INFORMATION FOR YOUR GP

- Moderate acute asthma, no intensive care needed.
- Inhaled steroid stepped up from Clenil to Fostair 100/6. Please review in 2 working days.
- Ibuprofen and aspirin allergy confirmed and recorded.

Signed: Dr P. Example, Internal Medicine Trainee, on behalf of Dr M. Sample, Consultant Respiratory Physician
Bleep 0009 (fictional) · Thursday 22 October 2026, 1:05 pm

Checked by ward pharmacist:
S. Example (fictional), 22 October 2026

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